Eastside Community Health Clinic AI use policy

Draft for board review · Effective when the board approves it · Owner: Executive director, until the board names an owner · Next review: In 12 months

Why we have this policy. We want AI to give providers their evenings back. We must protect patient information and keep people in charge of care.

Who it covers. All staff, volunteers, contractors and board members.

TOOLS WE USE

- **AI scribe from our health record vendor:** Three pilot providers only. Covered by our business associate agreement; may hold patient visit information.
- **New tools, add-ons and AI features.** Ask before adding any tool, plugin or AI feature. Nadia, privacy and security officer, approves anything touching patient information, after a business associate agreement. Beth approves the rest.
- **Accounts.** Use your work email, not a personal account. Turn off vendor training on what you type.

WHAT YOU CAN USE AI FOR

- **Everyone, now.** Any AI tool, signed in with work email, for work with no information about a real person.
- Look up general clinical questions with no patient information.
- Draft patient education handouts for clinician review.
- Draft letters like appointment reminders and sliding fee letters, with no names.
- Pilot providers only: AI scribe drafts visit notes.

NEEDS APPROVAL FIRST

- New use of patient information in an approved tool — *Nadia, privacy and security officer*
- New clinical use — *Dr. Okafor, medical director*
- AI billing or coding help — *Your manager*
- AI translation of patient materials — *Your manager*

OFF LIMITS

- **Never put this into an AI tool** unless the tool is approved for it above: Diagnoses, medications, behavioral health notes, substance use treatment records, HIV status, reproductive health, immigration status, sliding fee income documents, insurance numbers, staff files, the donor list, and passwords. HIPAA covers patient records; substance use records carry extra federal protection.
- Nobody uses AI to make clinical decisions.
- Nobody uses AI to decide sliding fee eligibility.

CHECK EVERYTHING BEFORE IT GOES OUT

- Read every word of a scribe note before signing. AI once added a medication we decided against.
- Check AI work for facts, names, numbers and tone. The sender is responsible.
- A clinician reviews AI handouts. Check AI billing codes.

DECISIONS ABOUT PEOPLE

- AI may help look something up or draft a note. A person decides.

BEING OPEN ABOUT AI

- Tell patients when the scribe is listening. They can say no.
- Reviewed handouts need no AI label. Chatbots must say they are not people.

IF SOMETHING GOES WRONG

- Tell Nadia right away if patient information may have gone into an AI tool.
- Nadia runs the HIPAA breach process. Reporting mistakes is not punished.

WHO OWNS THIS POLICY

- The executive director owns this until the board names someone.
- Managers go over it with new staff.
- The compliance committee reviews changes; the board approves them.

OPEN QUESTIONS FOR THE BOARD

- Do we roll the scribe out to all providers after the pilot, in about three months?
- Should behavioral health providers use the scribe at all?
- Do we pay for or formally approve tools staff already use, like ChatGPT, OpenEvidence and UpToDate AI search?
- What do we tell funders like HRSA and the state about our AI use?

Sample policy for an invented organization, drafted by Fulton Ring from a mock 30-minute call. Not legal advice; not reviewed by a real nonprofit.